

MediPort — Final Pitch

5-minute pitch script + 8-slide deck

Pitch Strategy

Target delivery: ~4:15–4:30, leaving room for pauses and Q&A. **Structure:**

Problem → Evidence → Solution → Advantage → Feasibility & Business → Ask **Core message:**

MediPort helps patients turn scattered medical records into something they can actually use in a consultation.

The Script# MediPort — Short Pitch Script

Slide 1 — The Problem

Hi, I'm Echo.

I have NF2, a rare disease. And I lost my hearing because of it.

Let me show you a problem I face at hospital visits.

I may have years of records, from different hospitals, in paper reports and MRI films.

At the same time, I may be trying to read the doctor's lips.

So the problem is simple:

My records are scattered. But I have to connect them.

Slide 2 — Why It Matters

And this is not just my problem.

I surveyed 38 NF2 patients and caregivers.

Zero had a tool to bring their records together.

66% still use paper or films.

71% reported problems communicating with a new doctor.

So the problem is not just storing records.

It is **using them across hospitals.**

Slide 3 — The Solution

That's why I'm building **MediPort**.

The idea is simple:

Capture. Organise. Share.

After a visit, I take a photo of my records.

MediPort reads the key information and builds one health timeline.

Before my next visit, I can create a short summary for my doctor.

And each key result links back to the original record.

Slide 4 — The Value

The key change is this:

I don't have to tell my whole story. I can show it.

The doctor can see my history without rebuilding it from a conversation.

For me, written information also makes communication easier.

The record becomes part of the conversation.

Slide 5 — Why MediPort?

Why not use a cloud drive?

Because a cloud drive stores files.

Why not use a hospital app?

Because it may only show one hospital's records.

MediPort connects the information across hospitals.

It is:

Patient-owned. Easy to follow. Built for the consultation.

Slide 6 — Can We Build It?

Yes.

The technology already exists.

Camera. OCR. AI. Timeline. Summary.

My first version will focus on one thing:

Turn messy records into a clear, trusted timeline.

And MediPort does not diagnose.

It only helps organise the patient's information.

Slide 7 — Business & Scale

I would start with NF2.

Then expand to other rare diseases and people who use many hospitals.

The model could be:

Free basic use, with paid AI features and family sharing.

There is also potential to work with patient organisations and insurers.

So:

NF2 is the starting point, not the limit.

Slide 8 — The Ask

My next step is simple:

Test MediPort in a real consultation.

I want to work with:

Patients. Clinicians. Rare-disease groups.

And I am looking for Monash's help with partners and a pilot.

My goal is simple:

Turn a workaround I built for myself into a tool that can help many people.

Thank you.

The Deck — 8 Slides

Slide 1 · The Five-Minute Problem

On screen

Five minutes.

Years of medical history.

Visual

Use a real photo of your **plastic bag of medical reports / MRI films**. Preferably your actual records rather than stock photography.

Purpose

Start with your lived experience. Do **not** introduce MediPort yet. The audience should first understand:

"This person has a real problem."

Slide 2 · The Gap

On screen

The patient connects the hospitals.

Visual

A simple diagram:

Hospital A ↓ PATIENT ↓ Hospital B ↓ PATIENT ↓ Hospital C

Show the hospitals as separate systems.

The patient is the connection between them.

Evidence

Keep this compact:

0 / 38 had a tool 66% paper / film 71% communication problems

You can mention the 76% hearing-loss statistic verbally rather than putting four statistics on the slide.

Purpose

This slide establishes the system-level problem, not just your personal story.

Slide 3 · MediPort

On screen

Collect first.

Organise later.

Visual

Three large hi-fi screens:

CAPTURE → TIMELINE → SUMMARY

Use your actual Figma prototype.

Important

Do not put paragraphs explaining each step.

Let the interface demonstrate the workflow while you explain it verbally.

Judging criteria

Desirability + Feasibility + Innovativeness

Slide 4 · The Payoff

On screen

I don't narrate.

I show.

Visual

Make the doctor-ready Summary screen the hero.

One subtle callout:

Every key number ↓ Original record

Optional interaction

If your prototype supports it, demonstrate:

Summary ↓ Tap result ↓ Original MRI / report

This is probably the strongest product demonstration in the whole pitch.

Purpose

This is the emotional and functional climax.

The judges should understand immediately:

"Oh — this changes the consultation."

Slide 5 · Why MediPort?

On screen

Built for the gap between hospitals.

Visual

Minimal comparison:

Hospital App	Cloud Drive	Manual Record App	MediPort
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Across hospitals — ✓ ✓ ✓ Timeline — — ✓ ✓ Consultation summary — — — ✓ Accessibility-first — — — ✓

Keep it extremely clean.

No paragraphs.

No logos unless necessary.

Purpose

Make the Innovativeness argument explicit.

Don't make judges figure out why your product is different.

Slide 6 · Feasibility

On screen

The technology already exists.

Visual

Camera ↓ OCR ↓ AI Extraction ↓ Timeline ↓ Summary

At the bottom:

Organise information. Never diagnose.

Optional small text:

MVP focus: reliable, traceable records.

Purpose

Answer:

"Can this actually be built?"

without wasting time explaining technical architecture.

Slide 7 · Business & Scale

On screen

NF2 is the starting point.

Visual

NF2 ↓ Rare Disease ↓ Chronic Care ↓ Multiple Hospitals

Underneath:

Subscription + Patient Organisations + Insurer Sponsorship

Purpose

Show Viability and scalability without making unsupported market-size claims.

Your key argument is:

Start narrow because NF2 gives you a clearly defined problem and user group.

Expand because the underlying problem is not unique to NF2.

Slide 8 · The Ask

On screen

Next:

Test it in a real consultation.

Visual

Three steps:

PATIENTS ↓ CLINICIANS ↓ RARE-DISEASE ORGANISATION

At the bottom:

Seeking: Clinical partner · Patient-org pilot · Incubation support

Your name and contact details can sit at the very bottom.

Purpose

End with a specific invitation, not simply "Thank you."

This directly addresses the challenge's final recommendation about how the university/judges can participate.

Design Rules

- 8 slides only.

- Do not use all 20 slides just because 20 is allowed.
- One idea per slide.
- One strong sentence rather than paragraphs.
- Use your own medical records and hi-fi prototype.
- Avoid stock photography.
- Minimum body text: 24pt
- Headline: 40–60pt
- Strong contrast.
- Keep the slides readable from the back of the room.
- Use the same visual language as your existing MediPort prototype.

Visual hierarchy

The strongest slides should be:

Slide 1 → Slide 3 → Slide 4 → Slide 8

These are:

Problem → Product → Payoff → Ask

Delivery Notes

1. Don't aim for exactly five minutes

Aim for around:

4:15–4:30

This gives you room for:

- pauses
- audience reaction
- slight variation in speaking speed
- a clean ending

Do not try to fill every second.

2. Pause at two moments

After:

"Zero had a tool."

Pause.

And after:

"I don't have to narrate my medical history. I can show it."

Pause.

These are your two strongest lines.

Let them land.

3. Don't read the slides

Your slides should work like visual anchors.

For example, Slide 3 only says:

Collect first. Organise later.

You explain the three steps.

The judges look at the product.

That is much stronger than giving them a paragraph to read.

4. Accessibility during Q&A

After "Thank you", if necessary:

"I lip-read, so during Q&A, please face me when you speak."

That's enough.

Don't over-explain your accessibility needs.

Likely Q&A

1. How is this different from existing health-record apps?

Most of them work within one provider. MediPort is designed for the gap between providers, where the patient has to connect the information.

2. What if the AI gets something wrong?

The summary is traceable. Important values link back to the original record, so the patient or clinician can verify them.

If they push further:

For uncertain extraction, the system should ask the user to review it rather than silently present it as fact.

3. Is this a medical device?

The initial product is designed to organise and present the patient's own information. It doesn't diagnose or recommend treatment.

If they push further:

Regulatory requirements would still need to be assessed as the product develops.

Don't say "we stay outside medical-device regulation." That's too absolute.

4. Why would people pay?

That's something I still need to validate. My initial hypothesis is a low-cost subscription, with caregivers and organisations as potential payers.

Then stop.

Don't try to prove willingness to pay before you've tested it.

5. Can you build this?

I'm building and validating the UX prototype. The MVP would need engineering support, which is part of what I'm looking for next.

This is a much stronger answer than pretending you already have the engineering resources.

The Three Things You Need to Prove

Don't try to prove everything in five minutes.

The judges only need to leave the room believing three things:

1. The problem is real.

The patient is currently connecting fragmented healthcare systems.

2. MediPort directly changes the consultation.

I don't narrate. I show.

3. You know what needs to happen next.

Test it with clinicians in a real five-minute consultation.

If those three things land, the rest of the pitch supports them.